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Tentative Ruling

PLAINTIFF SONIA ESPARZA'S MOTION

TO QUASH, OR IN THE ALTERNATIVE TO MODIFY, DEFENDANTS' DEPOSITION SUBPOENAS FOR PRODUCTION OF BUSINESS RECORDS DIRECTED TO ST. JUDE HERITAGE MEDICAL GROUP (MEDICAL), ST. JUDE HERITAGE MEDICAL GROUP (BILLING), AND ST. JUDE PROVIDENCE (RADIOLOGY DEPARTMENT); REQUEST FOR MONETARY SANCTIONS is TBD; court is inclined to DENY the motion as to Medical because Plaintiff does not address how records custodian generally cannot reasonably separate only those records that may relate to the neck, back, or headaches; the court is inclined to GRANT the motions as to Radiology and Billing. No sanctions will be imposed upon Defendants.

Background

This personal injury action arises from an automobile collision which occurred on May 14, 2024.

On October 31, 2025, Plaintiff Sonia Esparza filed suit against Defendants Gabriela Hernandez and Alicia Hernandez for (1) Negligence and (2) Negligence Per Se.

On January 5, 2026, Defendants filed their answer.

On April 2, 2026, the instant motion to quash was filed.

On June 18, 2026, an opposition was filed. (Defendants do not seek monetary sanctions in opposition.)

On June 25, 2026, a reply was filed.

Discussion^[1]

Plaintiff moves for an order quashing, or in the alternative modifying, Defendants' deposition subpoenas for production of business records directed to: St. Jude Heritage Medical Group (Medical); St. Jude Heritage Medical Group (Billing); and St. Jude Providence (Radiology Department).

The motion is rather brief and straightforward: Plaintiff argues that she has placed only neck pain, back pain, and headaches in issue such that the subpoenas permit disclosure of years (starting from 2015) of unrelated medical, billing, and radiology information that has nothing to do with the injuries claimed here. Plaintiff proposes modifications already limiting records by body part and condition.^[2]

Defendants' principal point in opposition is that causation is a central issue in this case. Defendants dispute that this minor-impact accident (causing \$2,776 in property damage) caused or substantially contributed to Plaintiff's alleged injuries or need for surgical intervention. (Plaintiff is claiming future medical expenses in excess of \$160,000.) Defendants contend that her complaints are attributable, in whole or in part, to longstanding preexisting conditions since through they have learned that Plaintiff has a significant history of preexisting low back complaints and treatment dating back at least ten years before the subject accident (sciatica, sacroiliac joint dysfunction, and piriformis syndrome). Effectively, as "medical conditions do not exist in isolated body-part silos" and the "human musculoskeletal and neurological systems are interconnected" (opp. pp. 2-3), Defendants' main position is that restricting subpoenas to those body parts would improperly prevent Defendants from obtaining evidence relevant to evaluating alternative causes of Plaintiff's complaints (i.e., their defense). (Opp. p. 2:21-23; Opp. p. 10.)

Accordingly, the threshold issue is whether medical records of unrelated conditions are discoverable. The answer is no.

Normally, information about medical conditions entirely different from the injury sued upon are beyond the scope of discovery (not "relevant to the subject matter"). (See A. Scope of Discovery and Limitations, Cal. Prac. Guide Pers. Inj. Ch. 6-A (b) [6:36.2]; see also *Britt v. Superior Court* (1978) 20 Cal.3d 844, 862-864 [A patient tenders the issue of his physical health if he files an action for personal injuries but only as to information which relates to the claimed injuries].)

Defendants point to *Slagle v. Super. Court.* (1989) 211 Cal.App.3d 1309 to support their position that "A past condition seemingly unrelated to the injury in question may nonetheless be discoverable as relevant to other issues raised in the lawsuit." (Opp. pp. 7, 11.)[3] (The reply does not address *Slagle*.) But *Slagle* is factually distinguishable.

In *Slagle*, one of the defendants (alleged to have caused the accident) overheard the plaintiff "tell the doctor that he was blind in both eyes six months prior to the accident." (Id. at p. 1312.) The plaintiff filed a motion to quash defendant's subpoena of his medical records; the appellate court denied the writ. The appellate court interpreted Evidence Code section 999 "to include within its scope medical records of a plaintiff where the plaintiff's conduct is shown to be relevant to the issue of proximate causation if 'good cause for disclosure' is shown." (Id. at p. 1314.) Applying that principle, the *Slagle* court reasoned that the defendants "showed good cause to believe that the history of treatment to petitioner's eyes was relevant to the cause of the accident." (Id. at p. 1315.) But here proximate causation is not being disputed. Proximate cause or legal exists if the actor's conduct (here, Plaintiff) is a substantial factor in bringing about the harm. (See e.g., *Lombardo v. Huysentruyt* (2001) 91 Cal.App.4th 656.) Here, however, there are no allegations that Plaintiff caused her own injuries from this accident.

The next authority Defendants cite to is *Dodson v. J. Pacific, Inc.* (2007) 154 Cal.App.4th 931, 935-936, fn. 2 for the proposition that "Evidence of prior injuries to impeach a personal injury plaintiff's credibility are relevant and admissible." (Opp. p. 8.) The reply does not address this argument nor *Dodson*. But the relevancy of that case is unclear as the dicta regarding the admissibility of evidence had to do with impeaching credibility at trial, not the discoverability of certain

evidence.

Defendants' reliance upon *Vinson v. Superior Court* (1987) 43 Cal.3d 833 is also not instructive. (The reply cites to *Vinson* but does not address/analyze the case.) In *Vinson*, the plaintiff filed suit for sexual harassment in the workplace. The defendants' actions were alleged to have caused her to suffer continuing emotional distress. (Id. at p. 837.) The defendants moved for an order compelling her to undergo a medical and a psychological examination to test the true extent of her injuries and to measure her ability to function in the workplace. (Id. at p. 838.) The plaintiff opposed the motion as a violation of her right to privacy. (Ibid.) The court disagreed reasoning that the plaintiff's "present mental and emotional condition is directly relevant to her claim and essential to a fair resolution of her suit; she has waived her right to privacy in this respect by alleging continuing mental ailments." (Id. at p. 842.) If anything, *Vinson* undercuts Defendants' argument as the *Vinson* defendants also sought to probe into the plaintiff's sexual history and practice but as to that, the appellate court said no. "[S]he has not, merely by initiating this suit for sexual harassment and emotional distress, implicitly waived her right to privacy in respect to her sexual history and practices. Defendants fail to explain why probing into this area is directly relevant to her claim and essential to its fair resolution." (Id. at p. 842.)[4]

All in

all, while the

court acknowledges that the body is "interconnected," that does not allow defendant to obtain unlimited discovery of a plaintiff's medical records in order to ferret out a possible defense to an action for damages.

Notwithstanding, records custodian cannot reasonably separate body parts. Accordingly, Plaintiff's modification is impractical. This critical point is not addressed in reply.

The only subpoenas which may be subject to quash are the billing and radiology records.

As for billing records, Defendants appear to justify the unrestricted billing subpoena as a means of identifying other medical providers and verifying Plaintiff's medical history. (Opp. at pp. 4-5, 11.) However, as noted in reply, Defendants already have that information since Plaintiff already disclosed her providers under oath, her verified repose to Form

Interrogatory No. 6.4 identifies her providers, treatment, dates of care, and charges. (Reply p. 4.)

As for radiology, this point is not squarely addressed in reply. As set forth in opposition, Defendants seek these records because “neighboring structures, including the spine, nerve roots, pelvis, and supporting musculature relevant to the injuries at issue” since “[s]uch evidence is directly relevant where the plaintiff claims aggravation of a preexisting condition.” (Opp. p. 3, emphasis added.) But generally, an injured person is entitled to recover full compensation for all damages that proximately result from a defendant's tortious act, even if some or all of the injuries might not have occurred but for the plaintiff's preexisting physical condition, disease, or susceptibility to injury i.e., the eggshell skull rule.

Conclusion

Based on the foregoing, the court is inclined to grant the motion as to the Billing and Radiology subpoenas but deny the motion as to Medical.

[1] Defendant also objects to the filing of one motion when it is directed at three subpoenas. Defendants are correct in that general motion practice is that one motion should be directed at one form of relief. (See Rutter Group, California Practice Guide: Civil Procedure Before Trial ¶9:24.3 (2024) [practice for combining motions].) That said, as the arguments would be identical, the court declines to deny the motion on said grounds.

[2] (For clarity, Plaintiff does not dispute that “Defendants are entitled to records concerning her neck, back and headache complaints, including prior similar complaints to those body parts.” (Reply p. 2:4-5.)

[3] Defendants also cite to Evidence Code section 999 which creates an exception to the physician-patient privilege for “a communication relevant to an issue concerning the condition of the patient in a proceeding to recover damages on account of the conduct of the patient....”

[4] Defendants also cite to McClatchy Newspapers v.

Superior Court (1945) 26 Cal.2d 386 for their proposition that information pertinent to a potential claim or defense is discoverable. That case did not involve the right to privacy with respect to medical records; the case was an action for libel.